

Missing Patient Flow Chart Quick Reference Guide

Adult / Child patient missing from ward/department – unable to contact
Preliminary actions undertaken (appendix 1)
Local security have been notified LRI ext. 6767. GH ext. 2999. LGH ext. 4292
Duty manager, Nurse in charge, Patients medical team informed

Has the patient 'absconded'? i.e. are they detained or likely to be detained under the Mental Health Act, the MCA Deprivation of Liberty Safeguards, or are they subject to a public health order or subject of custody (Police or Prison) Or are under 18 years of age ?

Is the patient at risk of immediate threat to life and limb as a result of being missing?

Does the patient lack mental capacity to consent to / decline care and treatment? This should be documented or based on a reasonable belief

Is the person likely to suffer serious harm, including being at risk of exploitation or abuse, or cause serious harm to another person as a result of going missing

Do you believe the patient wishes / intends to take their own life?

NO

YES

Be prepared to provide a clear rationale to the Police
Ensure you have access to senior medical advice.
(Consultant or Registrar)
DON'T DELAY IF YOU THINK THERE IS A RISK
NURSE IN CHARGE TO CALL THE POLICE

LOW RISK

HIGH RISK

Outcome: Low Risk

1. Ring patients telephone number on their notes
2. Inform Nurse in Charge, Matron, Duty manager, security and consultant
3. Unable to locate or make contact with the patient, or the patient refuses to return to hospital? Complete discharge letter and notify the GP, named contact and other relevant professional agencies, district nurse, social worker etc.
4. Document all your actions and assessments in the patient's notes.
5. If you receive new information that may change the outcome review the flowchart and **escalate**.

Outcome: High Risk

1. Notify the Police advising them of the risk level and your concerns.
2. The Police will ask you the **questions overleaf**, be prepared to have the information available.
3. The police will want to speak to the last people (doctor and nurse) that saw the patient. They may need to remain in the area or leave a telephone number to speak to the Police.
4. Contact patients named contact
5. Liaise with clinical staff throughout regarding clinical concerns of the patient.
6. Inform the Duty Manager who will escalate to Director on call & CMG HoN
7. Document your actions in patient's notes and complete DATIX incident form ticking the safeguarding box.

Details of officer taking the report:		
Patient details including Full name, DOB, Address, Telephone numbers: Preferred Name / Nickname		
(INSERT PATIENT STICKER)		
Brief circumstances of patient going missing:		
Where last seen	When last seen	By whom?
Individual's description. – Including clothing		
Steps taken so far to trace the individual. (Preliminary actions undertaken)		
Is this out of character	YES - why	NO
Why are you concerned?		
What was the particular reason for this patient being deemed high risk?		
Any known intentions or preparations made prior to going missing?	YES - what	NO
Have they taken any personal items with them?	YES - what	NO
Do they have a mobile phone with them?	Network	Number
Any known places they may go?		
Are they subject to any mental health section	YES - What and Why	NO
Any specific medical needs that require medication	YES - Effects and timescales if not available	NO
Are they likely to be a victim of crime	YES - Why	NO
Are they likely to be a victim of abuse	YES (DV/Sexual/Racial/bullying/Homophobic)	NO
Are they at risk of sexual exploitation or on the Child protection register	YES – From whom	NO
Are they likely to self-harm, attempt suicide	YES – Give details, including last known attempt	NO
Have they been exposed to harm in any previous missing episode	YES - When	NO
Do they pose a danger to themselves or any other persons	YES	NO
Does the missing person have a current or previous history of drug or alcohol abuse	YES – Give details	NO
Details of any vehicle using or normal mode of transport if none		
Details of messaging and social media used	What format and usernames and passwords if known	
What access do they have to money		
Is there any other information relevant to their absence that may affect or influence a supervisor's risk assessment?		

MISSING PATIENT FOUND REPORT (MR2) Post Incident Joint Agency Debrief Form (High Risk only)

University Hospitals of Leicester 
NHS Trust

<u>Patient Sticker</u>	Ward / Department: Site: Date and Time Patient Found: Police reference number:
<u>Found address:</u>	<u>Officer Details:</u>
DEBRIEF	
Missing Voluntarily? Yes / No	Lost or Injured? Yes / No
Found at: Returned of own accord / Found at home address / Found at relative address / Found elsewhere <i>Describe circumstances:</i>	
Found by: Family / Police / carers / Other persons / Arrested / Recovered warrant MHA / Other <i>Describe circumstances:</i>	
Transport for return: Police provided / Carers provided / Not required / Ambulance required <i>Describe circumstances:</i>	
Suffered harm? Unharmd / Physical injury / Sex Offence / Other crime / Abduction / Unexplained death / Murder <i>Describe circumstances:</i>	
Circumstances While Missing: Slept rough / Stayed with friend / Met up with other / Went to location / Went to place previously lived or frequented / Stayed in hotel or other commercial premises / Misper refused or declined information / Not known / other <i>Describe circumstances:</i>	
Criminal activity: No crime committed / committed crime <i>Describe circumstances:</i>	
Time spend missing: Less than 24 hours / 24-48 hours / 3-5 days / More than 7 days <i>Describe circumstances:</i>	
Distance away from point missing: 0-5 miles / 6-210 miles / 11-20 miles / 21-40 miles / 41-80 miles / over 81 miles / out of UK <i>Describe circumstances:</i>	
Reason for going missing: Problems at home / personal problems / problems at school / other reasons / declined to give reasons / Lacks mental capacity to provide reasons <i>Describe circumstances:</i>	
Lessons Learnt	
Recommendation	